

COMPOSITION:

Each tablet contains 500mg Calcium Carbonate

PRODUCT DESCRIPTION

A white to off-white, round tablet, with marking 'd' at both side.

PHARMACODYNAMICSMechanism of actions:Antacid:

Calcium Carbonate reacts chemically to neutralise or buffer existing amounts of stomach acid but has no direct effect on its output. This action results in increased pH value of stomach contents, hence providing relief of hyperacidity symptoms. It also reduces acid concentration within the lumen of the oesophagus, thus causing an increase in intra-esophageal pH and a decrease in pepsin activity, which helps in the control of gastro-esophageal reflux.

Supplementary:

Intake of recommended dosage of calcium carbonate results in increased intestinal calcium absorption. This action results in decrease of calcium deficiency. Calcium carbonate, also binds phosphate in the gastrointestinal tract to form insoluble complexes and reduces phosphate absorption.

PHARMACOKINETICSAbsorption:

Calcium carbonate is converted to calcium chloride by gastric acid (hydrochloric acid) in the stomach, which results in the formation of carbon dioxide and water. Calcium is then absorbed to the extent of about 15-25% from the intestines but the majority is reconverted into insoluble calcium salts such as carbonate and stearate which is excreted in the faeces.

Distribution, Metabolism and Excretion:

Once absorbed from the stomach, physiological concentrations of calcium are tightly controlled, principally through the effects of parathyroid hormone, vitamin D and its metabolites and calcitonin. These control mechanisms are well documented in standard texts.

INDICATIONS

Recommended for the symptomatic relief of acid indigestion, heartburn, flatulence and nausea also as a supplementary source of calcium when normal requirements are high and in the correction of calcium deficiency in the diet. They can be used in osteoporosis therapy as an adjunct to more specific conventional treatments. It is also used as phosphate binder to treat patients with chronic kidney disease, renal failure and renal osteodystrophy.

DOSAGE AND ADMINISTRATION

To be taken orally, with a full glass of water with or before meals.

Adults, children over 12 years and elderly:

Antacid:

500 mg or 1000mg (1 or 2 tablet) 3 to 4 times a day after meals

Hypocalcaemia:

1.25g (2 ½ tablets, i.e equivalent of 500mg of calcium) 2 to 4 times a day after meal

Hyperphosphatemia:

5 -13g (10 -26 tablets, i.e equivalent of 2 to 5.2g of calcium) per day in divided doses with meals

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

1. Patients on low phosphate diets, or receiving cardiac glycosides or with impaired hepatic or renal function.
2. Contraindicated in hypersensitivity to any of the ingredients
3. Contraindicated in hypercalcaemia, hypercalciuria (e.g. some forms of malignant disease), and calcium renal calculi

WARNINGS AND PRECAUTIONS

1. If symptoms persist consult your doctor or pharmacist.
2. Use with caution in diseases associated with hypercalcaemia such as sarcoidosis and some malignancies and in patients with a history of renal calculi.
3. In renal insufficiency the tablets should be given only under controlled conditions for hyperphosphataemia. Caution should be exercised in patients with a history of renal calculi.
4. For long-term treatment, it is advisable to monitor serum, urinary calcium levels and kidney function. To consider reduce or stop treatment temporarily if urinary calcium exceeds 7.5mmol/24 hours (300mg/24 hours).

INTERACTIONS WITH OTHER MEDICAMENTS

1. Calcium enhances the effects of digital glycosides on the heart and may precipitate digitalis intoxication.
 2. Calcium salts reduce the absorption of certain drugs including tetracyclines and phenytoin, because of possible formation of nonabsorbable complex. Patients should be advised not to take calcium supplements within 1 to 3 hours taking the medications.
 3. Thiazides diuretics may increase the renal reabsorption of calcium, thereby leading to hypercalcemia.
 4. Interactions may be minimised by ensuring not taking any calcium supplements at the same time as, nor within three hours of other medications.
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PREGNANCY AND LACTATION

Data on a large number of exposed pregnancies indicate no adverse effects of 500 mg calcium carbonate tablets on pregnancy or on the health of the fetus/newborn child. To date no other epidemiological data are available.

Calcium carbonate can be used during lactation if taken as instructed. However, no problems would be anticipated from the use of this product during lactation, if taken in accordance with the posology. However, in humans, long term hypercalcaemia can lead to physical and mental retardation, aortic stenosis and retinopathy in a new born child.

SIDE EFFECTS

Calcium Carbonate can cause constipation and flatulence. Hypercalcaemia can occur as can alkalosis following the regular use of Calcium Carbonate. The milk-alkali syndrome has occasionally occurred in patients taking large doses. 'Acid Rebound' has been reported on cessation of Calcium Carbonate.

SYMPTOMS AND TREATMENT OF OVERDOSE

Symptoms, may include anorexia, nausea, vomiting, constipation, abdominal pain, muscle weakness, mental disturbances, polydipsia, polyuria, nephrocalcinosis, renal calculi and, in severe cases, cardiac arrhythmias.

In such cases of hypercalcaemia, it is advised to remove source of calcium. Rehydration may be necessary (if necessary with intravenous 0.9% sodium chloride) and a loop diuretic may be given to increase urinary calcium excretion.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

None known

PACK SIZE

Blister pack of 10 x 10 tablets, 50 x 10 tablets and 100 x 10 tablets

STORAGE

Store in a dry place below 30°C.

Keep out of reach of children / Jauhi dari kanak-kanak

PRODUCT REGISTRATION HOLDER

Duopharma Manufacturing (Bangi) Sdn. Bhd.

Lot No. 2, 4, 6, 8 & 10, Jalan P/7,
Section 13, Bangi Industrial Estate,
43650, Bandar Baru Bangi, Selangor, Malaysia

MANUFACTURER

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